Name: Shawon Sarkar | DOB: 10/29/1985 | MRN: U5426032 | PCP: Tanya T Smith, MD | Legal Name: Shawon Sarkar

Routine Visit - Feb 16, 2026

with Christy Gibson at UW Medicine Urgent Care at Kirkland

Do you need to make corrections to these records? Request an update.

Notes from Care Team

Progress Notes

Christy Gibson at 2/16/2026 1:20 PM

I was present with the medical student. I personally performed the physical examination and medical decision making. I have verified all of the medical student's documentation for this encounter.

Here for maroon colored stool
Stool was of soft consistency (mashed potatoes)
No fever
FHX of IBD in mother
LLQ pain reported, tenderness present on exam without rebound
Rectal exam notable for empty vault, no masses or hemorrhoids
Borderline POC WBC
DDX includes Upper GI bleed, diverticulitis, IBD, Colon CA, infectious diarrhea
Will get CT
Stool studies ordered if stools become looser
Referral for colonoscopy

Christy Gibson, MD

Kaden L at 2/16/2026 1:20 PM

Subjective:

Shawon Sarkar is a 40 year old female presenting for

Chief Complaint

Patient presents with

- Abdominal Pain
  C/o LLQ abd px like a sharp twinge which started closer to belly button x 4 days Px is sporadic. Blood in stool x AM 2/16
  Hx of chronic constipation w/ slow BM.

Intermittent LUQ pain starting 4 days ago
This morning she noticed maroon stool this morning.
She endorses hs of chronic constipation. Since her fibroid surgery she has had darker softer stool. She notes that she has been trying to eat more vegetables and yogurt. Last wk she became worried about her lisinopril causing hyperkalemia and then switched to a low K diet.
6 mo ago she underwent fibroid removal. She notes that the sx she is experiencing are similar to when she had her large (9.7cm) fibroid.

Notes previous relief with BM or passing flatus.

No Fhx of cancer. Mom with IBD w/ uveitis.

Review of Systems:

Patient Active Problem List

Diagnosis

- Elevated blood pressure reading
- Phobia, flying
- Iron deficiency anemia

Medical History

Past Medical History:

Diagnosis	Date
• Anemia	2023
• Fibroids *Had surgery last month*	2024
• Hypertension	2023
• NEGATIVE PAST MEDICAL HISTORY OF *Fibroids*	2023

Surgical History

Past Surgical History:

Procedure	Laterality	Date
• PR MYOMECTOMY 1-4 MYOMAS 250 GM/< VAGINAL APPR		8/14/25

Outpatient Medications Prior to Visit

Medication	Sig	Dispense	Refill
• ergocalciferol (Vitamin D-2) 1.25 mg (50,000 unit) capsule	Take by mouth.		
• lisinopril 10 MG tablet	Take 1 tablet (10 mg) by mouth daily.	90 tablet	0
• LORazepam 0.5 MG tablet	Take 1 tablet (0.5 mg) by mouth 2 times a day as needed for anxiety.	10 tablet	0
• MAGNESIUM CITRATE OR	Take 1 tablet by mouth daily.		
• Omega-3 Fatty Acids (OMEGA 3 OR)	Take by mouth.		
• ondansetron 4 MG disintegrating tablet	Dissolve 1 tablet (4 mg) on top of tongue and swallow. (Patient not taking: Reported on 2/16/2026)		
• Prenatal Vit-Fe Fumarate-FA (Prenatal Vitamin) 27-0.8 MG tablet	Take 1 tablet by mouth daily.	90 tablet	3
• pseudoephedrine ER 120 MG 12 hr tablet	Take 1 tablet (120 mg) by mouth every 12 hours as needed for congestion. (Patient not taking:	10 tablet	0

	Reported on 2/16/2026)
• VITAMIN D OR	Take by mouth.

Facility-Administered Medications Prior to Visit

Medication	Dose	Route	Frequency	Provider	Last Rate	Last Admin
• COVID-19 Pfizer mRNA vaccine (Comirnaty) injection 30 mcg	30 mcg	Intramuscular	Once	Smith, Tanya Tinette, MD		
• influenza trivalent PF vaccine syringe 0.5 mL	0.5 mL	Intramuscular	Once	Smith, Tanya Tinette, MD		

Review of patient's allergies indicates:

Allergies

Allergen	Reactions
• Varicella Virus Vaccine Live	Skin: Rash

Social History

Social History

Tobacco Use

- Smoking status: Never
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Not Currently
  Comment: Rarely
- Drug use: Never

Objective:

BP (!) 143/88 Comment: pt reports feeling anxious | Pulse 100 | Temp 37.1 °C (Temporal) | Resp 18 | LMP 02/14/2026 (Exact Date) | SpO2 96%

Gen: anxious
HEENT: deferred
Neck: supple. No adenopathy. Thyroid symmetric, normal size, without nodules
Heart: PMI normal. No lifts, heaves, or thrills. RRR. No murmurs, rubs or gallops
Lungs: deferred
Abdomen: inspection: nondistended and symmetric
auscultation: normal bowel sounds
palpation: soft, nontender, no palpable masses, no organomegaly, no guarding or rebound tenderness
Extremities: Normal, without deformities, edema, or skin discoloration, radial and DP pulses 2+ bilaterally
Skin: Skin color, texture, turgor normal. No rashes or concerning lesions
Neuro: Grossly normal to observation, gait normal
Rectal exam: rectal normal, no masses and guiaic negative stool. The patient was asked if they would like to have a chaperone in the room for this exam. The patient declined a chaperone.

Labs ordered, reviewed and interpreted by provider. Results:

Results for orders placed or performed in visit on 02/16/26 (from the past 12 hours)
POC Whole Blood Hemoglobin

Collection Time: 02/16/26 2:20 PM

Result	Value	Ref Range
Hemoglobin, POCT	12.7	11.5 - 15.5 g/dL

WBC, Onsite

Collection Time: 02/16/26 2:20 PM

Result	Value	Ref Range
WBC, POC	9.5	4.0 - 10 10*9/L

Imaging ordered, reviewed, and interpreted by provider. Official read by radiology pending. Provisional results KUB: Moderate stool burden. No transition point to suggest obstruction.

Assessment / Plan

		ICD-10-CM	
1.	**Blood in stool**	**K92.1**	**XR Abdomen 1 View** **POC Whole Blood Hemoglobin** **WBC, Onsite** **Colonoscopy - Screening & Surveillance** **CT Abdomen And Pelvis w Contrast** **Enteric Pathogens Panel**

Differential to include IBD, malignancy, diverticulitis, infectious cause. Guaiac in clinic (-) today. No s/s to suggest acute pathology. Referral for colonoscopy placed. CT order placed.

2.	Family history of inflammatory bowel disease	Z83.79	Colonoscopy - Screening & Surveillance CT Abdomen And Pelvis w Contrast
3.	LLQ abdominal pain	R10.32	CT Abdomen And Pelvis w Contrast

Kaden Lee
Medical Student

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